جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : سناء شعبان محمد علي
المحافظة : المنيا **الحي / المدينة :** العدوي **العنوان :** صفانية - العدوة - المنيا
رقم البطاقة : 28308242402863 **رقم التليفون :** 01159750510 **تاريخ تسجيل الإستمارة :** 24-10-2026
العمر(تاريخ الميلاد) : 24-08-1983 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: سناء شعبان محمد علي
National ID: 28308242402863

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستاف زوسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصري من أجل صحة مصر

MDT FORM 14/9/2026 G

Name	سناء شحاته محمد	MRN	٢٠٢٥٠٧٥٤
Age	٤١	ID	٢٨٢٠٨٤٤٠٢٨٦٣
Menopausal status		Phone	٠١١٠٩٧٠٠٥١٠
NRS SCORE		Marital status	
BMI		Family history	-
CO morbidity		REASON OF MDT	Post op

Clinical Data

BX:

ER:	PR:	HER 2:	Ki67:
8/8	8/8	+3	40 %

Sonomammography:

received NACT (T-Carbo) TCHP + HP

Mets Workup:

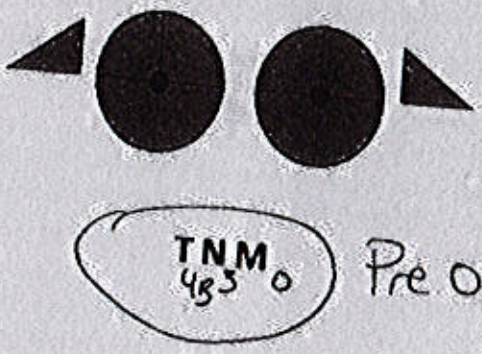
6 cycles TC

4 cycles Phesgo

Last cycle 14/1

left MRM

R L



Recommendations: ↓

Post op Staging → ypT1b , ypN1

MDT OUTCOME & PLAN

Adj Radio 6 X 2 weeks

+ AI = Zoladex X 3 years

+ PORTIC

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: سناء شعبان محمد على	
MR. : 2872	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

The patient is a 42-year-old with triple-positive breast cancer (TPBC) undergoing treatment with left mastectomy and reconstruction (LT MRM) performed in August 2025. The current systemic regimen includes KADCYLA (ado-trastuzumab emtansine) combined with ARAMIDEX (anastrozole) and ZOLADEX (goserelin), with a port placed for infusion. An echocardiogram performed on 25-12-2025 showed a left ventricular ejection fraction of 67 %. The patient received KADCYLA cycles C6, C7, and C8 as documented.

During a clinical pharmacy visit on 28-02-2026, the patient was noted to be receiving cycle C7 of KADCYLA plus ARAMIDEX, with an echocardiogram EF of 67 %.

On 21-03-2026, the clinical pharmacy note confirmed continuation of cycle C8 of KADCYLA and ARAMIDEX, with the same EF of 67 % and laboratory work reviewed and accepted.

On 11-04-2026, the oncology team recorded that cycle C7 was not administered due to a pharmacy issue ("qarar"). The plan was to revise the pharmacy order and proceed with cycle C8 of KADCYLA after labs, together with ARAMIDEX, supportive care, and ZOLADEX. Dental assessment for ZOMETA was pending.

Later on 11-04-2026, the pharmacy noted elevated liver enzymes (ALT 55 U/L, AST 73 U/L) and recommended liver support.

On 02-05-2026, the oncology note detailed the patient's weight (96 kg), diabetes, left-sided breast cancer (clinical stage T4 d, N3, M0, IDC grade 2, ER 8/8, PR 8/8, HER2 3+, Ki-67 40 %). The patient had received left MRM on 19-08-2025, with a port placed, and was on ZOLADEX and ARAMIDEX. Severe constipation, headache, and right calf muscle pain prompted a lower-limb venous duplex (negative) and brain MRI (negative). Laxatives were prescribed, and the next appointment was scheduled for cycle C9 of KADCYLA 260 mg, ARAMIDEX, and ZOLADEX, with a due date of 07-04-2026. Liver enzymes were noted to be two-fold elevated, prompting liver support.

On 23-05-2026, constipation resolved spontaneously, the lower-limb venous duplex remained negative, and brain MRI showed no space-occupying lesions. KADCYLA became available from pharmacy for continuation with ARAMIDEX and supportive care; dental assessment for ZOMETA remained pending.

On 03-06-2026, the patient was diagnosed with left temporal-lobe epilepsy; EEG performed on 05-06-2026 confirmed the diagnosis. Brain MRI remained free of lesions, and the ejection fraction was 70 % on 29-04-2026. The plan included cycle C10 of KADCYLA 260 mg, ZOMETA, ARAMIDEX, and continued liver support for three-fold elevated liver enzymes, with an abdominal ultrasound ordered.

تحريرا في ,

التاريخ :	18-09-2026	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

On 27-06-2026, the plan for cycle C11 of KADCYLA plus ARAMIDEX was outlined, pending laboratory results and liver support. Mildly elevated liver enzymes persisted. The patient continued treatment for temporal-lobe epilepsy with no contraindications to therapy. ZOLADEX was due on 04-07-2026.

On 19-07-2026, the patient was free of symptoms and scheduled for cycle C12 of KADCYLA 260 mg, ARAMIDEX, liver support, and ZOLADEX due on 26-09-2026.

On 16-08-2026, the current regimen includes ARAMIDEX with supportive care for three months, ZOLADEX due on 26-09-2026, and the next follow-up visit is planned for 08-11-2026.

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / سناء شعبان محمد على	Request No.	: 13003327
Patient No.	: 28308242402863	Request Date	: 19/08/2025
Path No.	: 2573/25	Verification Date	: 25/08/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 41 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer post NACT .
Nature of the specimen: Left breast & Left axilla (MRM)

Gross Examination:
- Procedure: Left MRM.

measured 25x20x8 cm with covering skin ellipse measured 18x15 cm .
- Tumor extent : A greyish white tumour bed site
- Size: 9 x7 x3 cm .
- Skin involvement: Nipple & areola were unremarkable grossly .
- Distance from margins: 2 cm away from deep margin .

Block designation :
1:9 TBS
10.Nipple
11.Deep margin
12:15 ALNDS
Grossing by Dr.Heba El-Shafeey .
- •Microscopic examination:
- Histological type: Invasive duct carcinoma (Actual tumour size 1x1 cm , Cellularity 35%) with medullary features.
- Grade (differentiation+ nuclear pleomorphism + mitosis): G 3(3+3+2)
- Ductal carcinoma in situ : Absent
- Skin involvement: Free nipple and areola
- Lymph node involvement: Positive L.Ns (1/10)
- Resection Margins: Free deep margin.
- Histologic response to therapy: Pathologic partial response

Diagnosis:
Left MRM (post NACT) :

Verified by :

CAP ACCREDITED
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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / سناء شعبان محمد على	Request No.	: 13003327
Patient No.	: 28308242402863	Request Date	: 19/08/2025
Path No.	: 2573/25	Verification Date	: 25/08/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 41 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Invasive duct carcinoma grade 3(Actual tumour size 1x1cm) , cellularity 35 %, with medullary features.

- Free deep margins
- Free nipple and areola
- Positive L.Ns (1/10) with therapy effect.
- Pathologic partial response
- Stage : ypT1b, ypN1
- RCB : 3.088 , RCB class II

Signature
Dr.Walaa Ibrahim Ghoneim
Dr.Heba El-Shafeey .

Verified by :

CAP ACCREDITED
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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / سناء شعبان محمد على	Request No.	: 13004130
Patient No.	: 28308242402863	Request Date	: 20/02/2025
Path No.	: 759/25	Verification Date	: 05/03/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 41 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

- PR: Positive (score 8/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 40% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Marwa Mahmoud.
Dr. Menna Nabil.

Verified by :
Dr.Toka Medhat

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / سناء شعبان محمد على	Request No.	: 13004130
Patient No.	: 28308242402863	Request Date	: 20/02/2025
Path No.	: 759/25	Verification Date	: 05/03/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 41 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast malignant looking mass.
Nature of Specimen: Tru-cut biopsy, collected at 20/2/2025.

Gross Examination:
- Three grayish white tissue cores up to 1.5 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
* Ductal carcinoma in situ: Focally positive.
* Perineural invasion: Negative.
* Angioinvasion: Negative.
* Microcalcifications: Positive.

* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- Her2: Strong membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 40% nuclear reaction of tumor cells.

Diagnosis:
Left breast malignant looking mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II, with focal DCIS and microcalification.
IPT:
- ER: Positive (score 8/8).

Verified by :
Dr.Toka Medhat

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
امانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Sanaa Shaaban Mohamed Ali	Date : 08/03/2025
Exam : C.T CHEST WITH CONTRAST	Age : 41 Year(s)
Medical ID: 754/2025	Pat. ID: 97818

POST CONTRAST MULTI-SLICE CT SCAN OF THE NECK, CHEST, ABDOMEN, AND PELVIS: -

I Neck

- Bilateral cervical prominent lymph nodes are seen the largest is seen at the left lower cervical measures about 1.8x1.1 cm.
- Normal appearance of the nasopharynx, oropharyngeal and laryngeal air spaces.
- No glottic, supra or infra glottic mass lesions.
- Intact cartilaginous skeleton of the larynx.
- Normal appearance of submandibular, parotid salivary glands.
- Normal size, enhancement & symmetrical appearance of thyroid glands.
- Normal appearance of the neck muscles & vessels.

II Chest:-

- Left breast diffuse ill defined parenchymal distortion with diffuse skin thickeing.
- Multiple left axillary level I & II pathological lymph nodes largest measuring 2 x 1.5 cm some of them show extracaspular invasion.
- Few left internal mammary enlarged lymph nodes largest measuring 0.8 x 0.5 cm.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

III Abdomen & pelvis:

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Sanaa Shaaban Mohamed Ali	Date : 08/03/2025
Exam : C.T CHEST WITH CONTRAST	Age : 41 Year(s)
Medical ID: 754/2025	Pat. ID: 97818

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

The examined skeleton shows:

- Heterogenous texture of the examine bones with mottled appearance.

Opinion:

- ***Left breast diffuse ill defined parenchymal distortion with diffuse skin thickeing.***
- ***Multiple left axillary level I & II pathological lymph nodes.***
- ***Few left internal mammary enlarged lymph nodes.***
- ***Heterogenous texture of the examine bones with mottled appearance? Metastatic rather than metabolic for bone scan correlation.***

Dr. Ali Saber, MSc.

Much obliged,

Dr. Reham Anwar, FEBR, MD

CT Scan
[[DOCTYPE:CT_SCAN]]

Mrs.Sanaa Shaban Mohamed

Date: 17/02/25

Ref. By Prof. Dr / Yasser Mohamed

BOTH BREAST ULTRASOUND REVEALED

RIGHT BREAST

- Normal breast parenchyma maintaining normal fibro glandular architectural pattern
- Normal ductal pattern and caliber .
- No dominant mass lesions seen
- Normal thickness of the skin.
- Normal nipple, areola complex.
- Right axilla show small accessory breast tissue , there are two small benign non-specific axillary lymph nodes the largest measures 16x7mm , oval in shape with preserved fatty hilum

LEFT BREAST

- There is ill defined hypoechoic heterogenous soft tissue mass seen at upper outer quadrant measures about 42x36mm , it show speculated outline.
  Small linear hypoechoic extension from the mass measures 35x7mm seen above the lesion
- Mild retro-areolar ductectesia .
- Diffuse thickened edematous skin
- Left axilla show two lymph nodes the largest measures 16x11mm , oval in shape with preserved fatty hilum

Conclusions

- Left breast mass likely malignant (BIRADS V)

Yours truly
Dr. Moustafa Farrag

Sonography
[[DOCTYPE:SONOGRAPHY]]

PATIENT NAME: SANA SHABAN MOHAMMED ALI
GENDER: Female
DATE : 13-Mar-2025 11:08 AM

PATIENT ID: 10125260285
AGE: 41 y
ORDER: 3290921

99MTC-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

No hot spot that account for osseous deposits could be detected allover the surveyed skeleton.
Degenerative changes are seen over dorso-lumbar vertebrae.
Arthritic changes are seen over both knees.
Both kidneys are normally visualized.

Conclusion:

No definite evidence of metastatic osseous deposits.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant **13-Mar-2025 02:18 PM**

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مركز بهية الهرم
١ شارع علوية أول شارع الهرم بجوار داري ، الجيزة
مستشفى بهية الشيخ زايد
٤ البلوك الأول - التسع رابط أمام كلية الهندسة
Bone Scan
[[DOCTYPE:BONE_SCAN]]